# Prodigy Anesthesia Quick Review

## Epiglottitis

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| What pathogen is responsible for the development of epiglottitis? | Haemophilus influenzae type B<br><br>*Barash PG, Cullen BF, Stoelting RK, Cahalan MK, Stock MC, Ortega R. Clinical Anesthesia. 7th ed. Philadelphia, PA: Lippincott Williams and Wilkins; 2013: 1364.* |
| What are the typical presenting signs in a patient with epiglottitis? | The most common presenting signs include sudden onset of fever, dysphagia, drooling, voice changes, and a preference for a seated position with the chest leaned forward and the head extended. If obstruction has developed, retractions and labored breathing may be present.<br>*Barash PG, Cullen BF, Stoelting RK, Cahalan MK, Stock MC, Ortega R. Clinical Anesthesia. 7th ed. Philadelphia, PA: Lippincott Williams and Wilkins; 2013: 1364.* |
| What age-group is most affected by epiglottitis? | Ages 2-7 are the most commonly affected, although the relative incidence has increased in the adult population as routine Haemophilus vaccination has decreased the incidence in the pediatric population.<br>*Hines RL, Marschall KE. Anesthesia & Co-Existing Diseases. 6th ed. Philadelphia, PA: Elsevier-Saunders; 2012: 615.* |
| In addition to clinical symptoms, what is one method of confirming a diagnosis of epiglottitis without manipulating the airway? | A radiograph of the soft tissue of the neck may reveal the thickened aryepiglottic folds and swelling of the epiglottis.<br><br>*Barash PG, Cullen BF, Stoelting RK, Cahalan MK, Stock MC, Ortega R. Clinical Anesthesia. 7th ed. Philadelphia, PA: Lippincott Williams and Wilkins; 2013: 1365.* |
| What precautions should be made in the evaluation of a patient with epiglottitis? | Because agitation and struggling can result in the dynamic collapse of the airway with subsequent total airway obstruction, care should be taken to keep the patient calm and relaxed. Excessive manipulation of the patient should be avoided and absolutely no attempt to evaluate the airway should be made in the unanesthetized patient.<br>*Barash PG, Cullen BF, Stoelting RK, Cahalan MK, Stock MC, Ortega R. Clinical Anesthesia. 7th ed. Philadelphia, PA: Lippincott Williams and Wilkins; 2013: 1365.* |
| What is the accepted induction and intubation technique for a pediatric patient with epiglottitis? | Inhalation induction with the child in the sitting position is acceptable. After the child becomes drowsy, he/she is placed supine and mask ventilated while intravenous access is acquired. Intubation is then performed with a styletted tube using an endotracheal tube that is 1/2 size smaller than normal.<br>*Hines RL, Marschall KE. Anesthesia & Co-Existing Diseases. 6th ed. Philadelphia, PA: Elsevier-Saunders; 2012: 614-615.* |

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